Haskins, Tim
Patient ID: **443887**
Specimen ID: **041-847-0771-0**

DOB: **05/14/1972**
Age: **53**
Sex: **Male**

Patient Report
Account Number: **09013070**
Ordering Physician: **R DEMARTINO**

labcorp

Date Collected: **02/10/2026** Date Received: **02/10/2026** Date Reported: **02/16/2026** Fasting: **Yes**

Ordered Items: **CMP14+LP+TP+TSH+5AC+CBC/D/P...; Cardiovascular Report; Litholink CKD Program; Insulin; Venipuncture; W. Charleston ,NV 702-878-4217**

Date Collected: **02/10/2026**

CMP14+LP+TP+TSH+5AC+CBC/D/P...

	Test	Current Result and Flag		Previous Result and Date	Units	Reference Interval
	Chemistries [01]					
▲	**Glucose** [01]	**151**	**High**		mg/dL	70-99
▲	**Hemoglobin A1c** [01]	**9.6**	**High**		%	4.8-5.6
	Please Note: [01]					
		Prediabetes: 5.7 - 6.4 Diabetes: >6.4 Glycemic control for adults with diabetes: <7.0				
	Uric Acid [01]	8.2			mg/dL	3.8-8.4
				Therapeutic target for gout patients: <6.0		
▲	**BUN** [01]	**29**	**High**		mg/dL	6-24
▲	**Creatinine** [01]	**1.64**	**High**		mg/dL	0.76-1.27
▼	**eGFR**	**50**	**Low**		mL/min/1.73	>59
	BUN/Creatinine Ratio	18				9-20
	Sodium [01]	139			mmol/L	134-144
	Potassium [01]	3.9			mmol/L	3.5-5.2
	Chloride [01]	101			mmol/L	96-106
	Carbon Dioxide, Total [01]	20			mmol/L	20-29
	Calcium [01]	9.8			mg/dL	8.7-10.2
▲	**Phosphorus** [01]	**4.3**	**High**		mg/dL	2.8-4.1
	Magnesium [01]	2.1			mg/dL	1.6-2.3
	Protein, Total [01]	7.5			g/dL	6.0-8.5
	Albumin [01]	4.6			g/dL	3.8-4.9
	Globulin, Total	2.9			g/dL	1.5-4.5
	Bilirubin, Total [01]	0.4			mg/dL	0.0-1.2
	Alkaline Phosphatase [01]	51			IU/L	47-123
▲	**LDH** [01]	**258**	**High**		IU/L	121-224
▲	**AST (SGOT)** [01]	**57**	**High**		IU/L	0-40
	ALT (SGPT) [01]	44			IU/L	0-44
	GGT [01]	25			IU/L	0-65
	Iron Bind.Cap.(TIBC)	344			ug/dL	250-450
	UIBC [01]	247			ug/dL	111-343
	Iron [01]	97			ug/dL	38-169
	Iron Saturation	28			%	15-55
	Ferritin [01]	369			ng/mL	30-400
	Cholesterol, Total [01]	146			mg/dL	100-199
	Triglycerides [01]	117			mg/dL	0-149
▼	**HDL Cholesterol** [01]	**37**	**Low**		mg/dL	>39
	VLDL Cholesterol Cal	21			mg/dL	5-40

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Date Collected: **02/10/2026**

CMP14+LP+TP+TSH+5AC+CBC/D/P... (Cont.)

LDL Chol Calc (NIH)	88		mg/dL	0-99
T. Chol/HDL Ratio	3.9		ratio	0.0-5.0
Please Note:[01]				

```
                    T. Chol/HDL Ratio
                              Men  Women
1/2 Avg.Risk  3.4    3.3
    Avg.Risk  5.0    4.4
 2X Avg.Risk  9.6    7.1
 3X Avg.Risk 23.4   11.0
```

▲ **C-Reactive Protein, Cardiac**[01]	**3.01**	**High**	mg/L	0.00-3.00

```
Relative Risk for Future Cardiovascular Event
Low                <1.00
Average       1.00 - 3.00
High               >3.00
```

▲ **Homocyst(e)ine**[01]	**17.6**	**High**	umol/L	0.0-14.5
TSH[01]	2.090		uIU/mL	0.450-4.500
Thyroxine (T4)[01]	7.1		ug/dL	4.5-12.0
T3 Uptake[01]	30		%	24-39
Free Thyroxine Index	2.1			1.2-4.9
Triiodothyronine (T3)[01]	91		ng/dL	71-180
Triiodothyronine (T3), Free[01]	2.7		pg/mL	2.0-4.4
Reverse T3, Serum[A, 02]	15.7		ng/dL	9.2-24.1
T4,Free(Direct)[01]	1.37		ng/dL	0.82-1.77
Thyroid Peroxidase (TPO) Ab[01]	16		IU/mL	0-34
Thyroglobulin Antibody[03]	<1.0		IU/mL	0.0-0.9

```
Thyroglobulin Antibody measured by Beckman Coulter Methodology
It should be noted that the presence of thyroglobulin antibodies
may not be pathogenic nor diagnostic, especially at very low
levels. The assay manufacturer has found that four percent of
individuals without evidence of thyroid disease or autoimmunity
will have positive TgAb levels up to 4 IU/mL.
```

Vitamin D, 25-Hydroxy[01]	37.3		ng/mL	30.0-100.0

```
Vitamin D deficiency has been defined by the Institute of
Medicine and an Endocrine Society practice guideline as a
level of serum 25-OH vitamin D less than 20 ng/mL (1,2).
The Endocrine Society went on to further define vitamin D
insufficiency as a level between 21 and 29 ng/mL (2).
1. IOM (Institute of Medicine). 2010. Dietary reference
   intakes for calcium and D. Washington DC: The
   National Academies Press.
2. Holick MF, Binkley NC, Bischoff-Ferrari HA, et al.
   Evaluation, treatment, and prevention of vitamin D
   deficiency: an Endocrine Society clinical practice
   guideline. JCEM. 2011 Jul; 96(7):1911-30.
```

Fibrinogen Activity[01]	341		mg/dL	193-507
.[01]				
CBC, Platelet Ct, and Diff[01]				
WBC[01]	6.2		x10E3/uL	3.4-10.8
RBC[01]	5.39		x10E6/uL	4.14-5.80

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Date Collected: **02/10/2026**

CMP14+LP+TP+TSH+5AC+CBC/D/P... (Cont.)

Hemoglobin [01]	16.2		g/dL	13.0-17.7
Hematocrit [01]	48.8		%	37.5-51.0
MCV [01]	91		fL	79-97
MCH [01]	30.1		pg	26.6-33.0
MCHC [01]	33.2		g/dL	31.5-35.7
RDW [01]	13.1		%	11.6-15.4
Platelets [01]	216		x10E3/uL	150-450
Neutrophils [01]	56		%	Not Estab.
Lymphs [01]	29		%	Not Estab.
Monocytes [01]	8		%	Not Estab.
Eos [01]	6		%	Not Estab.
Basos [01]	1		%	Not Estab.
Neutrophils (Absolute) [01]	3.4		x10E3/uL	1.4-7.0
Lymphs (Absolute) [01]	1.8		x10E3/uL	0.7-3.1
Monocytes(Absolute) [01]	0.5		x10E3/uL	0.1-0.9
Eos (Absolute) [01]	0.4		x10E3/uL	0.0-0.4
Baso (Absolute) [01]	0.1		x10E3/uL	0.0-0.2
Immature Granulocytes [01]	0		%	Not Estab.
Immature Grans (Abs) [01]	0.0		x10E3/uL	0.0-0.1
. [01]				
Urinalysis Gross Exam [01]				
Specific Gravity [01]	1.024			1.005-1.030
pH [01]	5.5			5.0-7.5
Urine-Color [01]	Yellow			Yellow
Appearance [01]	Clear			Clear
WBC Esterase [01]	Negative			Negative
Protein [01]	Negative			Negative/Trace
▶ **Glucose** [01]	**3+**	**Abnormal**		Negative
Ketones [01]	Negative			Negative
Occult Blood [01]	Negative			Negative
Bilirubin [01]	Negative			Negative
Urobilinogen,Semi-Qn [01]	0.2		mg/dL	0.2-1.0
Nitrite, Urine [01]	Negative			Negative
Microscopic Examination [01]	Microscopic not indicated and not performed.			

Cardiovascular Report

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
Interpretation [04]	Note Supplemental report is available.			
PDF [04]	Not applicable			

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Date Collected: **02/10/2026**

Litholink CKD Program

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
Interpretation [04]	Note			

```
--------------------------------
CHRONIC KIDNEY DISEASE:
EGFR, BLOOD PRESSURE, AND PROTEINURIA ASSESSMENT
We presume eGFR has been less than 60 mL/min/1.73mE2 on at
least two occasions spaced at least 3 months apart. Current
eGFR is 50 mL/min/1.73mE2 corresponding to CKD stage 3a.
Potassium is within goal, 3.9 mmol/L. Glycemic control (HB
A1c: 9.6 %) is not within goal and additional action is
indicated.
EGFR, BLOOD PRESSURE, AND PROTEINURIA TREATMENT SUGGESTIONS
-
Guidelines recommend treating patients with type 2 diabetes
and CKD with an SGLT2 inhibitor for cardiorenal protection.
Guidelines recommend a target blood pressure of 120/80 mmHg
or less to reduce cardiovascular risk and CKD progression.
Assessment of albuminuria (urine albumin:creatinine ratio or
urine protein:creatinine ratio preferred) is recommended at
least annually in CKD patients for staging and disease
prognosis.
EGFR, BLOOD PRESSURE, AND PROTEINURIA FOLLOW-UP
-
Spot Urine Panel is recommended by guidelines, at least
yearly; Hemoglobin A1C within 3 months; fasting Renal Panel
within 1 month;
BONE and MINERAL ASSESSMENT
Phosphorus is above goal, 4.3 mg/dL. Calcium is within goal,
9.8 mg/dL. Carbon Dioxide is below goal, 20 mmol/L. Vitamin
D is adequate (37.3 ng/mL).
BONE and MINERAL TREATMENT SUGGESTIONS
-
Restrict dietary phosphate intake, especially from processed
foods, soft drinks, and convenience foods. If phosphate
binder is in use, increase dose; otherwise, begin one pill
with each substantial meal. Discontinue or reduce dose of
active vitamin D (calcitriol and its analogs) for CKD, if in
use. Guidelines suggest use of active vitamin D only if
calcium is within the lower half of the reference range and
phosphorus is normal. If not on alkali, begin sodium
bicarbonate, one 650 mg pill 2-3 times daily, otherwise
increase dose.
BONE and MINERAL FOLLOW-UP
-
25-Hydroxy Vitamin D within 12 months; fasting Renal Panel
within 1 month; fasting PTH with Renal Panel is recommended
by guidelines, at least yearly;
LIPIDS ASSESSMENT
LDL-C is normal, 88 mg/dL. Triglyceride is normal, 117
mg/dL. Non-HDL Cholesterol is normal, 109 mg/dL. HDL-C is
low, 37 mg/dL.
LIPIDS TREATMENT SUGGESTIONS
-
Therapeutic lifestyle changes are always valuable to
maintain optimal blood lipid status (diet, exercise, weight
management). If at least a 50% LDL reduction from baseline
```


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Date Collected: **02/10/2026**

Litholink CKD Program (Cont.)

```
has not been achieved, begin or increase statin. Consider
measurement of LDL particle number or Apo B to adjudicate
need for further LDL lowering therapy. If statin cannot be
tolerated or increased, alternatives include use of an
intestinal agent (ezetimibe or bile acid sequestrant) or
niacin.
LIPIDS FOLLOW-UP
-
fasting Lipid Panel within 12 months;
ANEMIA ASSESSMENT
Hemoglobin is normal, 16.2 g/dL. TSAT is within goal, 28.0
%. Ferritin is within goal, 369 ng/mL. Hemoglobin target
assumes ESA is not in use.
ANEMIA TREATMENT SUGGESTIONS
-
No specific change of treatment is indicated at this time.
ANEMIA FOLLOW-UP
-
CBC within 12 months;
------------------------------
DISCLAIMER
These assessments and treatment suggestions are provided as
a convenience in support of the physician-patient
relationship and are not intended to replace the physician's
clinical judgment. They are derived from national guidelines
in addition to other evidence and expert opinion. The
clinician should consider this information within the
context of clinical opinion and the individual patient.
SEE GUIDANCE FOR CHRONIC KIDNEY DISEASE PROGRAM: Kidney
Disease Improving Global Outcomes (KDIGO) clinical practice
guidelines are at http://kdigo.org/home/guidelines/.
National Kidney Foundation Kidney Disease Outcomes Quality
Initiative (KDOQI (TM)), with its limitations and
disclaimers, are at www.kidney.org/professionals/KDOQI. This
program is intended for patients who have been diagnosed
with stages 3, 4, or pre-dialysis 5 CKD. It is not intended
for children, pregnant patients, or transplant patients.
```

PDF [04]	.

Insulin

Test	Current Result and Flag	Previous Result and Date	Units	Reference Interval
Insulin [01]	7.7		uIU/mL	2.6-24.9

Previous Result
The Previous Result is listed for the most recent test performed by Labcorp in the past 5 years where there is sufficient patient demographic data to match the result to the patient. Results from certain tests are excluded from the Previous Result display.

Icon Legend
▲ Out of Reference Range ■ Critical or Alert

Footnotes/Disclaimers
A: This test was developed and its performance characteristics determined by Labcorp. It has not been cleared or approved by the Food and Drug Administration.

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Performing Labs
01: PDLCA - Labcorp Phoenix, 5005 S 40th Street Ste 1200, Phoenix, AZ 85040-2969 Dir: Earle Collum, MD
02: BN - Labcorp Burlington, 1447 York Court, Burlington, NC 27215-3361 Dir: Sanjai Nagendra, MD
03: SPOWA - Labcorp Spokane, 110 W Cliff Dr. Ste 100-200, Spokane, WA 99204-3614 Dir: Shefali Goyal, MD
04: LITNC - Labcorp Clinical / Digital, 10 Moore Drive, Durham, NC 27709-0009 Dir: Jennifer Ennis, MD
For inquiries, the physician may contact Branch: 800-762-4344 Lab: 800-788-9743

Patient Details
Haskins, Tim

Phone: **661-992-7005**
Date of Birth: **05/14/1972**
Age: **53**
Sex: **Male**
Patient ID: **443887**
Alternate Patient ID: **443887**

Physician Details
R DEMARTINO
PROFESSIONAL CO-OP SERVICES
2700 N 29th Ave Ste 308, Hollywood, FL, 33020

Phone: **866-999-4041**
Account Number: **09013070**
Physician ID: **1265468359**
NPI: **1265468359**

Specimen Details
Specimen ID: **041-847-0771-0**
Control ID: **00645977**
Alternate Control Number: **00645977**
Date Collected: **02/10/2026 0655 Local**
Date Received: **02/10/2026 0000 ET**
Date Entered: **02/10/2026 1118 ET**
Date Reported: **02/16/2026 0706 ET**

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SEE GUIDANCE FOR CARDIOVASCULAR REPORT: Grundy SM et al. 2018 Multisociety guideline on the management of blood cholesterol: a report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines. J Am Coll Cardiol 2019; 73: e285-350; Contois et al. Clin Chem 2009; 55(3):407-419; Brunzell et al. Diabetes Care 2008; 31(4):811-82.
SEE GUIDANCE FOR CHRONIC KIDNEY DISEASE PROGRAM: Kidney Disease Improving Global Outcomes (KDIGO) clinical practice guidelines are at http://kdigo.org/home/guidelines/. National Kidney Foundation Kidney Disease Outcomes Quality Initiative (KDOQI (TM)), with its limitations and disclaimers, are at www.kidney.org/professionals/KDOQI. This program is intended for patients who have been diagnosed with stages 3, 4, or pre-dialysis 5 CKD. It is not intended for children, pregnant patients, or transplant patients.

Note: *Please refer to your LabCorp Report for all results as well as any test-specific and specimen-specific comments.*

Chronic Kidney Disease

Analysis & Treatment Suggestions

GENDER	DIABETES	CURRENT eGFR	MOST RECENT CKD STAGE
M	Yes	**50**	**3a**

Prognosis of CKD by GFR And Albuminura Categories

GFR Categories: (ml/min/1.73m²)			A1	A2	A3
G1	>=90	NORMAL OR HIGH			
G2	60-89	MILDLY DECREASED			
G3a	45-59	MILDLY TO MODERATELY DECREASED			
G3b	30-44	MODERATELY TO SEVERELY DECREASED			
G4	15-29	SEVERELY DECREASED			
G5	<15	KIDNEY FAILURE			

Persistent Albuminura Categories

A1 = NORMAL TO MILDLY INCREASED; ACR <30 ug/mg; PCR <=200 mg/g
A2 = MODERATELY INCREASED; ACR 30-300 ug/mg; PCR 201-500 mg/g
A3 = SEVERELY INCREASED; ACR >300 ug/mg; PCR >500 mg/g

= LOW RISK = MODERATELY INCREASED RISK
= HIGH RISK = VERY HIGH RISK

● = PATIENT'S RESULT

Kidney Disease: Improving Global Outcomes (KDIGO) CKD Work Group. KDIGO 2012 Clinical Practice Guideline for the Evaluation and Management of Chronic Kidney Disease. *Kidney inter.; Suppl.* 2013; 3: 1-150.

eGFR, Blood Pressure, and Proteinuria

We presume eGFR has been less than 60 mL/min/1.73mE2 on at least two occasions spaced at least 3 months apart. Current eGFR is 50 mL/min/1.73mE2 corresponding to CKD stage 3a. Potassium is within goal, 3.9 mmol/L. Glycemic control (HB A1c: 9.6 %) is not within goal and additional action is indicated.

Treatment Suggestions

Guidelines recommend treating patients with type 2 diabetes and CKD with an SGLT2 inhibitor for cardiorenal protection. Guidelines recommend a target blood pressure of 120/80 mmHg or less to reduce cardiovascular risk and CKD progression. Assessment of albuminuria (urine albumin:creatinine ratio or urine protein:creatinine ratio preferred) is recommended at least annually in CKD patients for staging and disease prognosis.

Bone & Mineral

Phosphorus is above goal, 4.3 mg/dL. Calcium is within goal, 9.8 mg/dL. Carbon Dioxide is below goal, 20 mmol/L. Vitamin D is adequate (37.3 ng/mL).

Treatment Suggestions

Restrict dietary phosphate intake, especially from processed foods, soft drinks, and convenience foods. If phosphate binder is in use, increase dose; otherwise, begin one pill with each substantial meal. Discontinue or reduce dose of active vitamin D (calcitriol and its analogs) for CKD, if in use. Guidelines suggest use of active vitamin D only if calcium is within the lower half of the reference range and phosphorus is normal. If not on alkali, begin sodium bicarbonate, one 650 mg pill 2-3 times daily, otherwise increase dose.

Lipids

LDL-C is normal, 88 mg/dL. Triglyceride is normal, 117 mg/dL. Non-HDL Cholesterol is normal, 109 mg/dL. HDL-C is low, 37 mg/dL.

Treatment Suggestions

Therapeutic lifestyle changes are always valuable to maintain optimal blood lipid status (diet, exercise, weight management). If at least a 50% LDL reduction from baseline has not been achieved, begin or increase statin. Consider measurement of LDL particle number or Apo B to adjudicate need for further LDL lowering therapy. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant) or niacin.

Anemia

Hemoglobin is normal, 16.2 g/dL. TSAT is within goal, 28.0 %. Ferritin is within goal, 369 ng/mL. Hemoglobin target assumes ESA is not in use.

Treatment Suggestions

No specific change of treatment is indicated at this time.

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Follow-Up Suggestions for CKD

Recommended by guidelines, at least yearly

- Spot Urine Panel
- Fasting PTH with Renal Panel

1 month

- Fasting Renal Panel

3 months

- Hemoglobin A1C

12 months

- 25-Hydroxy Vitamin D
- Fasting Lipid Panel
- CBC

Cardiovascular Report

Patient Assessment

Current available clinical information suggests the patient's risk is at least HIGH. Your patient has laboratory results that suggest the presence of two CHD risk equivalents (diabetes and chronic kidney disease). Two additional major risk factors are present (age over 45 and HDL-C less than 40).

High sensitivity CRP and Lp(a) results identify sources of residual risk that may warrant more intensive therapy. Hs-CRP result (3.01 mg/L) indicates increased risk for future cardiovascular events.

Insulin resistance, obesity, excessive alcohol use, smoking, nephrotic syndrome, liver disease, and certain medications can cause secondary dyslipidemia. Consider evaluation if clinically indicated.

Therapeutic lifestyle changes are always valuable to achieve optimal blood lipid status (diet, exercise, weight management).

Lipid Management

Select one patient risk category based upon medical history and clinical judgment. Additional risk factors such as personal or family history of premature CHD, smoking, and hypertension modify a patient's goals of therapy. In CVD prevention, the intensity of therapy should be adjusted to the level of patient risk. MODERATE intensity statin therapy generally results in an average LDL-C reduction of 30% to less than 50% from the untreated baseline. Examples include (daily doses): atorvastatin 10-20 mg, rosuvastatin 5-10 mg, simvastatin 20-40 mg, pravastatin 40-80 mg, lovastatin 40 mg. HIGH intensity statin therapy generally results in an average LDL-C reduction of 50% or more from the untreated baseline. Examples include (daily doses): atorvastatin 40-80 mg and rosuvastatin 20 mg.

▽ = PATIENT'S RESULT

ANALYTE / RESULT	**Patient Risk Category** (select one)		
	LOW	**INTERMEDIATE**	**HIGH**
LDL-C **88 mg/dL**	160	100 130	70 100
non-HDL **109 mg/dL**	190	130 160	100 130
Lipid Assessment	LDL-C is optimal, 88 mg/dL. Non-HDL Cholesterol is optimal, 109 mg/dL.	LDL-C is optimal, 88 mg/dL. Non-HDL Cholesterol is optimal, 109 mg/dL.	LDL-C is normal, 88 mg/dL. Non-HDL Cholesterol is normal, 109 mg/dL.
Treatment Suggestions	Please refer to assessment and treatment suggestions under high risk category.	Please refer to assessment and treatment suggestions under high risk category.	If at least a 50% LDL reduction from baseline has not been achieved, begin or increase statin. Consider measurement of LDL particle number or Apo B to adjudicate need for further LDL lowering therapy. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant) or niacin.

DISCLAIMER: *These assessments and treatment suggestions are provided as a convenience and are neither comprehensive nor intended to replace the physician's clinical judgment. They do not include information such as family history, personal history, or physical findings as would be obtained by the clinician during patient evaluation because LabCorp does not have access to the complete patient medical record.*

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PATIENT	DATE OF BIRTH	GENDER	CKD STAGE	DATE OF SERVICE	PHYSICIAN
HASKINS, TIM	**05/14/1972**	**M**	**3a**	**02/10/2026**	LabCorp Account #: 09013070

Your Test Results

WITHIN GOAL | BORDERLINE | OUT OF GOAL

Test	Your Results	Comments
Blood Sugar		
A1C measures your average blood sugar control over the last 2-3 months.	**9.6%** (9.6%)	**YOUR A1C IS HIGH** which means your diabetes is poorly controlled. High blood sugar levels can damage your heart, blood vessels, kidneys, feet and eyes. Make sure you always take the medications prescribed by your doctor and follow his/her instructions.
Blood Lipids		
LDL CHOLESTEROL is the bad cholesterol that can clog your arteries.	**88** Ref. Range: 0 to 99 mg/dL	**HAVING LOW LDL HELPS PROTECT YOUR HEART AND BLOOD VESSELS.** Remember to take any medications prescribed by your doctor. Your doctor may recommend even lower cholesterol levels to reduce your risk of heart disease. Weight loss, exercise (at least 30 minutes 5 times a week), a diet low in trans and saturated fats, and quitting smoking can keep cholesterol low.
TRIGLYCERIDES are a type of fat and high levels may increase risk of heart disease.	**117** Ref. Range: 0 to 149 mg/dL	**YOUR TRIGLYCERIDES ARE NORMAL.** To keep them normal, limit the amount of alcohol, carbohydrates and sugars in your diet and keep your diabetes in control.
Kidney Health		
eGFR estimates how well your kidneys are filtering blood. The higher the number, the better your kidneys are working.	**50** Ref Range: >59 mL/min/1.73mE2	**YOUR RESULT COULD MEAN YOU HAVE STAGE 3a CHRONIC KIDNEY DISEASE (CKD).** There are 5 stages of CKD. See below for more information.
POTASSIUM helps keep your heart and muscles working properly. High or low levels can be dangerous.	**3.9** Ref. Range: 3.5 to 5.2 mmol/L	**YOUR POTASSIUM IS NORMAL.** To keep it in the normal range, make sure you avoid foods high in potassium. For more information, visit www.kidney.org/atoz/content/potassium.cfm.

DISCLAIMER: You should discuss this information with your physician. Labcorp does not have a doctor-patient relationship with you, nor does it have access to a complete medical history or physical examination conducted by a physician that would be necessary for a complete diagnosis and comprehensive treatment plan. Neither you nor your physician should rely solely on this guidance. Bolded result descriptions in "Comments" consider either the reference range or target range for the test result. Reference range refers to the Labcorp reference interval. Target range refers to the guideline-suggested goal. REFERENCES:National Kidney Foundation Kidney Disease Outcomes Quality Initiative (KDOQI) at www.Kidney.org and Kidney Disease Improving Global Outcomes (KDIGO) at http://kdigo.org. Adapted from: https://www.niddk.nih.gov/-/media/Files/Health-Information/Health-Professionals/Kidney-Disease/Your_Kidney_Test_Results_EN.pdf.American Diabetes Association's Standards of Medical Care in Diabetes-2022 (Diabetes Care, Vol 45, Supp 1, Jan 2022); National Diabetes Education Program's 4 Steps to Manage Your Diabetes for Life (2016, NIH publication 16-5492).

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PHOSPHORUS works with calcium to build strong, healthy bones and keep the rest of the body healthy. Too much phosphorus can damage the heart and blood vessels.

4.3

Ref. Range: 2.8 to 4.1 mg/dL

YOUR PHOSPHORUS IS HIGH. You should try to avoid or cut down on foods high in phosphorus. For more information, visit www.kidney.org/atoz/content/phosphorus.cfm. If you have been prescribed a phosphate binder, be sure to take it with meals.

HEMOGLOBIN is a test to look for anemia, or low blood count. Low hemoglobin levels may cause you to feel tired or have low energy.

16.2

Male Range: 13 to 17 g/dL

YOUR HEMOGLOBIN IS NORMAL. Because low hemoglobin is common and can happen over time, it is important to follow up regularly with your doctor for continued monitoring.

YOUR eGFR IS 50, YOU ARE IN STAGE 3a CKD. THINGS YOU NEED TO KNOW:

- CKD means your kidneys show decreased function which may be caused by aging, diabetes, high blood pressure, or many other diseases. CKD can lead to heart disease.
- Even if you feel fine, you may be at risk for problems such as high blood pressure, high cholesterol, anemia, and bone disease. Your doctor should test you for these problems.
- Avoid taking NSAIDs (e.g. ibuprofen, Advil®, Motrin®, Aleve®) as these may worsen your kidney function.
- To learn more, visit the National Kidney Foundation's website: www.kidney.org/kidneydisease.

DISCLAIMER: You should discuss this information with your physician. Labcorp does not have a doctor-patient relationship with you, nor does it have access to a complete medical history or physical examination conducted by a physician that would be necessary for a complete diagnosis and comprehensive treatment plan. Neither you nor your physician should rely solely on this guidance. Bolded result descriptions in "Comments" consider either the reference range or target range for the test result. Reference range refers to the Labcorp reference interval. Target range refers to the guideline-suggested goal. REFERENCES:National Kidney Foundation Kidney Disease Outcomes Quality Initiative (KDOQI) at www.Kidney.org and Kidney Disease Improving Global Outcomes (KDIGO) at http://kdigo.org. Adapted from: https://www.niddk.nih.gov/-/media/Files/Health-Information/Health-Professionals/Kidney-Disease/Your_Kidney_Test_Results_EN.pdf.American Diabetes Association's Standards of Medical Care in Diabetes-2022 (Diabetes Care, Vol 45, Supp 1, Jan 2022); National Diabetes Education Program's 4 Steps to Manage Your Diabetes for Life (2016, NIH publication 16-5492).

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